



Ethicare
Resourcing

NEW ZEALAND RESOURCE CENTRE

Interview Preparation Guide

For healthcare professionals preparing to interview
in New Zealand – across every profession.



WELCOME

You're probably more prepared than you think

If you're feeling unsure about interviewing in New Zealand, you're in good company. Most of the candidates we support share the same quiet worries: What do panels actually expect? Will my overseas experience be valued? What if I've never done a behavioural interview before?

These are reasonable questions — and every one of them has a clear answer. This guide walks you through it: what makes New Zealand interviews different, the culture behind them, how to structure your answers, and exactly what to do in the week before.

Most candidates we support are far more prepared than they realise. Often, it's simply a matter of understanding how to present the experience you already have.

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SECTION 01

What makes New Zealand interviews different

At Ethicare, we always say this: these interviews aren't designed to catch you out. They're a conversation – a chance for the team to understand how you think, how you care, and how you show up in practice. But they do place weight in slightly different places than you may be used to.

Values and behaviours come first

Panels want to understand how you work, not just what you've done. Expect questions about your judgement, your values, and how you treat people.

Behavioural questions are standard

You'll be asked for real moments from your own practice rather than hypothetical or textbook answers.

"Tell us about a time you supported a patient who was anxious or distressed..."

Examples over theory

Knowing the right answer matters less than showing you've lived it. Specific, honest examples always land better than polished generalities.

Communication style

Conversational, warm and direct. Panels respond well to candidates who speak plainly about what they did and why – including what was hard.

Cultural safety matters

Awareness of cultural safety, equity, and working respectfully with diverse communities is genuinely important. Openness counts more than expertise.

Reflection is a strength

Expect to talk about a time something didn't go to plan. Reflecting honestly on what you learned is seen as a strength, not a weakness.

"Tell us about a mistake you learned from..."

Whatever your profession – nursing, midwifery, allied health, medical imaging, anaesthetic technology, medicine or beyond – the same principles apply. Panels are choosing a colleague, not marking an exam.

SECTION 02

How healthcare works in New Zealand

A little context goes a long way in an interview. Here's the system you'd be joining, in brief.

Health New Zealand | Te Whatu Ora

Health New Zealand | Te Whatu Ora is the national organisation responsible for planning and delivering public health services. It was created in 2022, bringing the country's twenty district health boards together into one national system – so public hospitals and community services across New Zealand now sit within a single organisation, arranged into regions and districts.

Public healthcare is funded largely through taxation. Hospital and emergency care is free or low-cost for eligible residents, ACC (the Accident Compensation Corporation) covers care related to accidents and injuries, and primary care is delivered through general practices with part-charges. Private hospitals and clinics operate alongside the public system.

Current national priorities

- Improving access to care and reducing waiting times.
- Growing and supporting the health workforce – including international recruitment.
- Shifting more care into primary and community settings, closer to home.
- Achieving equitable outcomes, particularly for Māori and Pacific communities and rural populations.

Why this matters in your interview

Because services sit within one national system, panels are interested in how you'd work within a wider team and pathway – not just your own role. And because the workforce is genuinely international, they're practised at welcoming overseas clinicians: mentioning that you've looked into how the system works lands well and shows you've done your homework.

SECTION 03

Understanding New Zealand healthcare culture

Interviews reflect the workplaces behind them. Knowing how New Zealand teams work day to day will help you understand why panels ask what they ask – and how to answer naturally.

A flatter hierarchy

New Zealand healthcare teams tend to be less hierarchical than many systems overseas. First names are the norm – including for senior doctors and managers – and input is expected from every member of the team, regardless of seniority. If you've come from a more formal system, this can take adjusting to. In your interview, show that you're comfortable contributing your view while still respecting the team around you.

A speak-up culture

Patient safety relies on people raising concerns early – and New Zealand services actively encourage it. Panels often explore this directly: Would this person escalate a concern, even to someone senior? Would they speak up if something didn't look right? Prepare a real example of a time you questioned something, escalated a concern, or advocated for a patient. It's one of the most valued answers you can give.

Teamwork and whānau-centred care

Care in New Zealand is built around the patient and their whānau – their family and support network. Decisions are often made with families, not just delivered to them. Multidisciplinary teamwork carries real weight, so think of moments where you collaborated across professions or involved a family in care decisions.

Balance and wellbeing

Work-life balance is taken seriously, and teams look out for one another. It's perfectly acceptable – even positive – to talk about why lifestyle and wellbeing are part of your reason for choosing New Zealand. It signals you've thought the move through.

SECTION 04

Cultural safety, Māori health and Te Tiriti

This is the area international candidates worry about most – and the one where expectations are most commonly misunderstood. So let's be clear:

You are not expected to be an expert in Māori culture or Te Tiriti o Waitangi. You are expected to be open, respectful, and willing to learn — and to talk through personal examples of culturally safe care.

A brief orientation

- Te Tiriti o Waitangi (the Treaty of Waitangi) is New Zealand's founding document, signed in 1840 between the British Crown and Māori. In healthcare, it underpins a commitment to partnership, participation and protection — working with Māori to achieve equitable health outcomes.
- Cultural safety means care where the patient — not the clinician — defines whether they felt safe and respected. It asks you to reflect on your own assumptions and adapt your practice to each person.
- Whānau means family in its widest sense. Involving whānau in care and decisions is central to good practice.
- Equity is a priority across the system: recognising that different communities need different support to achieve the same outcomes.

SECTION 04 — CONTINUED

How to talk about it in your interview

Draw on what you've already done, wherever in the world you practised. Almost every clinician has examples of culturally safe care — they just haven't labelled them that way:

- Adapting communication for a patient whose first language wasn't yours, or working effectively with interpreters.
- Respecting religious or cultural needs around treatment, modesty, food, or end-of-life care.
- Involving family in decisions, or recognising when a patient's community shaped how care should be delivered.
- Noticing a gap between the care a group was receiving and the care they needed — and doing something about it.

A strong shape for your answer

"I haven't worked in New Zealand yet, so I'm still learning about Te Tiriti and Māori health models — and I'm genuinely looking forward to that. What I can tell you is how I approach cultural safety in my own practice..." followed by one specific, honest example. Humility plus a real story beats memorised facts every time.

SECTION 05

Behavioural questions and the STAR structure

Behavioural questions ask for real moments from your own practice. A simple structure like STAR helps panels follow your thinking – but what matters most is that it feels natural and authentic.

S**Situation**

Set the scene briefly. Where were you, and what was happening?

T**Task**

What needed to happen, and what was your responsibility?

A**Action**

What did you do, and why? This is where your judgement shows.

R**Result**

What was the outcome – and what did you take from it?

A worked example

Supporting a distressed patient

Situation: "A patient arrived for a procedure visibly distressed – she'd had a frightening experience during a previous appointment."

Task: "I needed to complete the procedure safely while making sure she felt in control throughout."

Action: "I talked her through every step beforehand, agreed a signal she could use to pause, and checked in with her throughout."

Result: "We completed it fully – her first time without sedation. I now use the same approach with every anxious patient."

Use examples like this as a guide, not a script. Panels can tell the difference between a memorised answer and a real story told naturally – keep your responses conversational and your own.

Prepare two or three situations like this from your own practice. Strong territory: supporting a colleague under pressure, escalating a concern, improving a workflow, adapting care for an individual patient. You don't need to be perfect – you need to be you, at your best.

SECTION 06

Questions we hear time and time again

No two interviews are identical, but these themes come up again and again – along with what the panel is really asking.

Q1 Why New Zealand?

WHAT THEY'RE REALLY ASKING

Genuine motivation – lifestyle, practice environment, family – and evidence you've thought it through. There's no wrong answer, only an unconsidered one.

Q2 Why this service or department?

WHAT THEY'RE REALLY ASKING

Show you've read about the service and the community it cares for. Even one or two specific observations signal real interest.

Q3 Tell us about yourself.

WHAT THEY'RE REALLY ASKING

A two-minute professional story: your background, what you care about in your practice, and why this move makes sense now. Practise it out loud.

Q4 Describe a challenging situation and how you handled it.

WHAT THEY'RE REALLY ASKING

A classic STAR moment. Choose something real, focus on your actions and judgement, and finish with what you learned.

Q5 Tell us about a mistake you learned from.

WHAT THEY'RE REALLY ASKING

Honesty and reflection are what's being tested. Pick a genuine example, own it plainly, and show how it changed your practice.

Q6 How do you prioritise under pressure?

WHAT THEY'RE REALLY ASKING

Walk through your actual decision-making – clinical urgency, safety, communication with the team – using a real busy shift as your example.

Q7 What does excellent patient care look like to you?

WHAT THEY'RE REALLY ASKING

Ground it in a real patient moment rather than a definition. Person-centred care, dignity and communication matter here.

Q8 How do you support your colleagues?

WHAT THEY'RE REALLY ASKING

Teamwork carries real weight in New Zealand. Think of a time you noticed a colleague struggling and quietly stepped in.

Q9 How would you raise a concern with someone senior?

WHAT THEY'RE REALLY ASKING

This is the speak-up culture question. Show you'd act early, respectfully and clearly — patient safety first, egos second.

Q10 What do you understand by cultural safety?

WHAT THEY'RE REALLY ASKING

Not a definition test. Acknowledge you're learning about the New Zealand context, then give a personal example of adapting your care respectfully (see Section 04).

Q11 Why should we choose you?

WHAT THEY'RE REALLY ASKING

Not a trap — an invitation. Summarise what you bring, connect it to their service, and let your enthusiasm be visible.

Q12 How do you cope with change, or working in a new or developing service?

WHAT THEY'RE REALLY ASKING

New Zealand services are often evolving, and panels want flexibility and initiative. Show you're proactive, comfortable with some uncertainty, and see shaping a service as an opportunity rather than a worry.

Q13 What's your current situation?

WHAT THEY'RE REALLY ASKING

Simply be honest and frame it positively. If you're between roles, mention what you've been doing — progressing registration, keeping your knowledge current, preparing for the move. Panels respect candour.

SECTION 07

Clinical questions — and the mistakes we see

Alongside behavioural questions, expect one or two clinical or scenario-based questions appropriate to your profession and scope — a deteriorating patient, a safety concern, a prioritisation dilemma, an equipment or medication issue. These aren't exams; panels want to hear how you think.

How to approach a clinical scenario

- Patient safety first. Start with the immediate priority and reason aloud, step by step.
- Escalate appropriately. Saying when you would call for help is a strength — it is the speak-up culture in action.
- Stay within your scope. Be clear about what you would do yourself and what you would refer on.
- It's fine not to know. "I would check the local protocol and confirm with a senior colleague" is a respected answer. Guessing is not.

Mistakes we see time and time again

Across many New Zealand interviews, the same avoidable mistakes come up:

- Generic answers. One of the most common mistakes we see is answers that could apply to any hospital in the world. Mention the service, the community, the role.
- Underselling autonomy. Candidates often tell us afterwards they didn't make their scope of practice clear. State plainly what you do independently.
- Memorising cultural safety. Reciting facts about Te Tiriti lands far worse than one honest, personal example of culturally safe care.
- Listing tasks, not judgement. Panels want to hear why you did things and what the outcome was — not a duty list.
- No questions for the panel. In our experience, candidates who ask nothing leave the impression they're interviewing anywhere. Prepare two or three.
- Time-zone slips. Confirm the interview time in writing, in both time zones.

SECTION 08

Preparing your own examples

Use this page to shape your two or three key stories. Choose moments that genuinely reflect who you are as a clinician – ordinary, human examples often land best.

Example 1 — a patient moment I'm proud of

SITUATION · TASK · ACTION · RESULT

Example 2 — a challenge, mistake or concern I handled

SITUATION · TASK · ACTION · RESULT

Example 3 — culturally safe or whānau-centred care

SITUATION · TASK · ACTION · RESULT

SECTION 09

Video interviews — and questions to ask the panel

Most international candidates interview by video call, often across time zones. A few practical habits make a real difference:

- Test your camera, microphone and connection the day before — and again an hour before.
- Double-check the time zone conversion. Confirm it in writing if you're unsure.
- Choose a quiet, well-lit spot with a tidy background; face a window if you can.
- Look at the camera when speaking — it reads as eye contact.
- Keep brief notes beside you (your three examples, your questions) — glancing is fine.
- If the technology fails, stay calm. Panels deal with it constantly; how you handle it says good things about you.

Questions worth asking the panel

Interviews run both ways — good questions show genuine interest and help you decide if the service is right for you:

- "How is a new international team member supported in their first few months?"
- "What does the orientation to Māori health and cultural safety look like here?"
- "How does the team approach professional development and further training?"
- "What do people enjoy most about working in this service — and living in this region?"

SECTION 10

Your week-before checklist

Everything to have ready in the seven days before your interview:

- Re-read the job description and person specification – note the values they name.
- Read about the service, the hospital or organisation, and the community it serves.
- Prepare your two-minute “tell us about yourself” answer and practise it aloud.
- Shape your three STAR examples (Section 08) – including one cultural-safety story.
- Prepare your speak-up example: a time you escalated or raised a concern.
- Think through your honest answer to “Why New Zealand?” and “Why this service?”
- Prepare two or three questions to ask the panel.
- Confirm the date, time and time zone in writing; test your video setup.
- Plan rest the night before – calm and genuine beats over-rehearsed.

You don't need to be perfect. You just need to be you — at your best.

SECTION 11

After the interview

A short, genuine thank-you email within a day of your interview is rarely expected from overseas candidates — which is exactly why it lands so well. Keep it brief, warm and specific.

A template you can adapt

Subject: Thank you — [Role] interview

Hi [Name],

Thank you very much for taking the time to meet with me today. I really enjoyed learning more about the service and the team, and [one specific thing that genuinely interested you — the service model, a development they mentioned, the team culture].

I'm very enthusiastic about the role and feel my experience in [your key strength] aligns closely with what the team is building. I'd value the opportunity to contribute.

Thank you again for your time and consideration — I look forward to hearing from you.

Kind regards,
[Your name]

What happens next

- Timeframes vary — a week or two is common, sometimes longer with multiple candidates across time zones. A polite follow-up after a week is perfectly acceptable.
- If Ethicare arranged your interview, we'll chase the outcome for you and pass on honest feedback either way.
- If it's a yes, offer and reference checks usually follow quickly — have your referees pre-warned and ready.
- If it's not, ask for feedback. New Zealand panels usually give it generously, and it sharpens the next interview.

HOW ETHICARE WILL SUPPORT YOU

You don't have to navigate this alone

If you choose to partner with Ethicare, we'll support you from CV preparation through to interview readiness — including a practice run before the real thing, guidance on the right examples to prepare, and honest feedback at every stage.

We've reviewed hundreds of international healthcare CVs. We've helped healthcare professionals relocate to New Zealand since 2023. And we understand both what candidates bring and what employers are looking for.

NEXT STEPS

1. Work through the worksheet in Section 08 and shape your three examples.
2. Book a discovery call — we'll talk through your situation, no obligation.
3. Explore the New Zealand Resource Centre for registration, immigration and moving guides.

Book a Discovery Call · hello@ethicareresourcing.com

ethicareresourcing.com

